

COMMON SYMPTOMS

- **Malar rash** or “butterfly” rash on the cheeks and bridge of the nose
- **Discoid rash** on the skin, a disc shaped rash often found on the face scalp or ears which can cause scarring
- **Rash or irritation on sun exposed skin**
- **Sores or ulcers in the mouth and nose**
- **Hair thinning/loss**
- **Raynaud’s syndrome**, or sudden loss of blood to peripheral joints
- **Red/purple spots on the fingertips**, which may indicate Raynaud’s or antiphospholipid syndrome
- **Tender and swollen joints**

TYPES OF LUPUS

- **Systemic Lupus Erythematosus (SLE)**, lifetime diagnosis requiring ongoing treatment
- **Drug-Induced lupus (DIL)**, short term lupus caused by drug usage
- **Cutaneous Lupus**, lupus manifested as skin inflammation
- **Neonatal Lupus**, lupus in newborns may occur if the mother has certain antibodies in her blood

Find a lupus support group:



Walk to End Lupus Indianapolis
October 17th, 2026:



Learn more about and
donate to lupus research:



Lupus awareness trifold:



Lupus awareness pamphlet:



DIAGNOSES & TREATMENT FOR SLE

An informal pamphlet
about Systemic Lupus
Erythematosus

TESTING

PHYSICAL EXAMS

A doctor may look for malar rash or “butterfly” rash, discoid rash, rash or irritation on sun exposed skin, sores or ulcers in the mouth and nose, hair thinning/loss, Raynaud’s syndrome, red/purple spots on the fingertips, and tender or swollen joints (see common symptoms for more detail).

BLOOD TESTING

Tests can reveal certain markers of disease like inflammation levels, rheumatoid factor, organ function, and antinuclear antibodies which can indicate the presence of immune disease, and other tests that may rule out other rheumatoid diagnoses.

OTHER TESTING

X-rays and ultrasounds can reveal tissue damage, determine the progression of disease, and to rule out other diagnoses.

TREATMENT

INITIAL TREATMENT

Depending on the progression of illness, initial treatment may vary.

Most patients will start on hydroxychloroquine immediately, and may be prescribed an NSAID for joint pain and inflammation. Doctors often recommend topical corticosteroids for rashes and suggest daily use of sunscreen and protective clothing and hats.

For those with more active disease, doctors may prescribe oral corticosteroids and Immunosuppressants.

For those with more major organ involvement, treatments might include high dose corticosteroids (sometimes intravenously), more aggressive Immunosuppressants, and close monitoring of the organs, including kidney biopsies for patients with lupus nephritis.

LONG TERM TREATMENT

A specialist will commonly have a lupus patient meet with them every 3-6 months to do routine physical exams and blood work, and to discuss any changes in the patient’s disease activity.

MEDICINAL

NSAIDS

Non-steroidal anti-inflammatory drugs, used for joint pain and general inflammation control.

CORTICOSTEROIDS

Used to control flares; ideally used for short-term aggressive treatment.

IMMUNOSUPPRESSANTS

Suppresses the immune system to regulate the creation and life span of disease fighting cells.

BIOLOGICS

Targets specific parts of the immune system (for lupus, b-cells), typically administered as a weekly shot or monthly intravenous infusion as they would break down in the stomach.